

[illegible]

1. Skin/together/joints first 80/10/10

WHAT YOU SEE

Poster: skin first 80%, together 10%, joints first 10% (10/80/10)

WHAT IT ENCODES

In most patients with PsA the skin psoriasis precedes the arthritis — roughly 80% develop skin disease first (often by ~10 years), ~10% present with skin and joints together, and ~10% have arthritis BEFORE any rash (psoriatic arthritis sine psoriasis). Overall ~30% of psoriasis patients eventually develop PsA. Severity of skin disease does not reliably predict joint disease, but nail involvement does. Mnemonic: skin leads the parade 80% of the time, but the 10% arthritis-first group is the board favorite because the diagnosis is missed.

2. Hidden scalp/nail psoriasis

WHAT YOU SEE

Magnifier labeled HIDDEN over hidden lesion

WHAT IT ENCODES

In arthritis-first or skin-poor PsA you must hunt for HIDDEN psoriasis in the scalp/hairline, behind the ears, umbilicus, intergluteal/natal cleft, perineum, and nails. A family history of psoriasis (first-degree relative) also counts toward CASPAR. Asking only 'do you have a rash?' will miss occult disease — physically examine these sites and the nails.

BOARD TRAP

A negative skin exam of the arms/legs does NOT exclude PsA — failing to check scalp, gluteal cleft, umbilicus and nails is the classic miss.

3. Wooden mannequin pattern

WHAT YOU SEE

Articulated wooden mannequin with letters S/M/U/G/M

WHAT IT ENCODES

PsA has five classic Moll & Wright patterns: (1) asymmetric oligoarthritis (≤4 joints, most common at presentation), (2) symmetric polyarthritis (RA-like), (3) DIP-predominant, (4) spondylitic/axial (sacroiliitis, spondylitis), and (5) arthritis mutilans (rare, destructive). Patterns overlap and evolve over time. The signature distinguishing PsA from RA is asymmetry, DIP involvement, dactylitis, and enthesitis.

4. RAY DIP finger

WHAT YOU SEE

Single finger lit up bottom-left labeled RAY

WHAT IT ENCODES

PsA characteristically involves ALL joints of a single digit ('ray' pattern — MCP+PIP+DIP of one finger) producing a sausage ray, and preferentially targets the DIP joints, which is exceptional in inflammatory arthritis. DIP disease tracks with adjacent nail disease (shared extensor enthesis). RF and anti-CCP are typically negative (seronegative spondyloarthritis).

BOARD TRAP

RA classically SPARES the DIP joints and involves MCP/PIP symmetrically — prominent DIP synovitis points away from RA and toward PsA (or DIP osteoarthritis).

5. Sausage foot dactylitis

WHAT YOU SEE

Swollen FEET! / sausage toe at top

WHAT IT ENCODES

Dactylitis ('sausage digit') is diffuse swelling of an entire finger or toe from combined synovitis AND flexor tenosynovitis — not just joint swelling. It is a hallmark of spondyloarthritis (PsA, reactive arthritis) and a marker of more erosive disease. Present in ~30–40% of PsA. The whole digit is uniformly swollen, blurring joint margins.

6. Tailor (chronic inflammation)

WHAT YOU SEE

Central tailor figure with inflamed red hands

WHAT IT ENCODES

PsA is a chronic inflammatory arthritis: morning stiffness >30–60 min, improvement with activity, soft-tissue swelling, and elevated ESR/CRP (though normal inflammatory markers do NOT exclude it). Enthesitis (inflammation at tendon/ligament insertions — Achilles, plantar fascia) is a core feature shared across the SpA family. RF and anti-CCP are negative in ~95%.

7. Nail pitting / 30%

WHAT YOU SEE

A magnified fingernail close-up labelled 30% showing pits and onycholysis — the pitted nail is the strongest clinical clue to DIP joint disease because nail bed and DIP extensor share one enthesis.

WHAT IT ENCODES

Nail disease occurs in ~80–90% of PsA (vs ~40% of psoriasis without arthritis) and includes pitting (>20 pits is significant), onycholysis, oil-drop/salmon patches, subungual hyperkeratosis, and ridging. Nail involvement is the strongest clinical predictor of DIP joint disease because the nail bed shares an enthesis with the DIP extensor tendon.

8. DIP / ASYM / SYM / AXIAL / MUTILANS rack

WHAT YOU SEE

Coat rack of shirts labeled DIP, ASYM, SYM, AXIAL, MUTILANS

WHAT IT ENCODES

The subtype 'wardrobe': DIP-predominant, ASYMmetric oligoarthritis, SYMmetric polyarthritis, AXIAL/spondylitic, and arthritis MUTILANS. Mutilans is the most destructive — osteolysis causes 'telescoping' or 'opera-glass' (main en lorgnette) digits where the finger collapses on itself. Axial PsA tends to be more asymmetric with chunky non-marginal syndesmophytes, unlike the symmetric fine syndesmophytes of ankylosing spondylitis.

9. CASPAR criteria

WHAT YOU SEE

Checklist CASPAR: psoriasis (2), nail, dactylitis, RF, X-ray, score ≥ 3

WHAT IT ENCODES

CASPAR classification (≥ 3 points in a patient with inflammatory articular/spine/enthesal disease): current psoriasis = 2 points; personal/family history of psoriasis = 1; psoriatic nail dystrophy = 1; negative RF = 1; current or prior dactylitis = 1; juxta-articular new bone formation on X-ray = 1. Specificity ~99%, sensitivity ~91%. It is a CLASSIFICATION (research) tool, not a strict diagnostic test.

10. DM RISK tag

WHAT YOU SEE

A hanging cardboard tag reading DM RISK pinned to the workshop wall — the tag flags the metabolic-syndrome/diabetes and cardiovascular comorbidity burden that travels with PsA.

WHAT IT ENCODES

PsA carries a high comorbidity burden: obesity, metabolic syndrome, type 2 diabetes, dyslipidemia, fatty liver, and accelerated cardiovascular disease (independent CV risk like RA). Also uveitis, IBD, and depression. These comorbidities influence drug choice (e.g., avoid methotrexate with fatty liver/alcohol) and require active CV risk-factor management.

11. Fluffy new bone / pencil-in-cup

WHAT YOU SEE

Label FLUFFY and dish of bits (PIC) at bottom center

WHAT IT ENCODES

Radiographic hallmark of PsA is the COEXISTENCE of erosion and proliferation: 'fluffy' periostitis/periosteal new bone, enthesophytes, and the pathognomonic 'pencil-in-cup' deformity — the eroded sharpened end of one phalanx (pencil) sits in the cupped expanded base of the next. Bone density is usually PRESERVED (no periarticular osteopenia). Arthritis mutilans shows gross osteolysis.

BOARD TRAP

RA produces ONLY erosions plus periarticular osteopenia (no new bone). The presence of fluffy periostitis / new bone formation alongside erosions, with preserved bone density, distinguishes PsA from RA.

12. OA hand

WHAT YOU SEE

Hand labeled OA at bottom right

WHAT IT ENCODES

Key mimic — DIP osteoarthritis: Heberden nodes (DIP) and Bouchard nodes (PIP) are bony, hard, NON-inflammatory enlargements with osteophytes and the 'gull-wing' erosion of erosive OA. No psoriasis, no nail pitting, no dactylitis, normal ESR/CRP. Pain is mechanical (worse with use, better with rest) — the opposite of inflammatory PsA stiffness.

BOARD TRAP

DIP swelling in an older patient is more often Heberden-node OA than PsA — the discriminators are absence of inflammatory stiffness, normal CRP/ESR, hard bony (not boggy) joints, and no skin/nail psoriasis.

13. PsA hand

WHAT YOU SEE

Inflamed hand labeled PsA at far bottom right

WHAT IT ENCODES

The PsA hand contrasts with the OA hand: soft, boggy inflammatory DIP synovitis, dactylitis, nail dystrophy, and a ray/asymmetric distribution with skin psoriasis. Treatment escalates by phenotype — NSAIDs for mild/oligoarticular and enthesitis; methotrexate/leflunomide/sulfasalazine for peripheral polyarthritis (DMARDs do NOT help axial disease); TNF inhibitors (adalimumab, etanercept, infliximab), IL-17 inhibitors (secukinumab, ixekizumab), IL-23/IL-12-23 inhibitors (guselkumab, ustekinumab), and JAK inhibitors (tofacitinib, upadacitinib) for refractory or axial disease.